Hope Hospital		
		Written Guidance on Self-Administration of Medication
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1. Purpose

To establish a clear and safe protocol for the **self-administration of medications** by patients during hospitalization or treatment under supervision, ensuring patient safety and compliance with NABH requirements.

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2. Scope

This policy applies to all inpatients and outpatients who are authorized to self-administer prescribed medications while under the care of the hospital or healthcare facility.

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3. Definition

Self-administration of medication refers to the act of a patient taking their prescribed medications by themselves, without direct administration by a nurse or physician, but under supervision or with prior approval from the treating team.

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4. Policy Statement

Self-administration of medications is allowed **only when**:

- It is **clinically appropriate and safe**
- The **treating physician authorizes it in writing**
- The patient is **capable, trained**, and willing to administer the medication correctly
- The medication is **prescribed** and part of the treatment plan

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5. Guidelines and Procedure

A. Eligibility Criteria

A patient may be considered for self-administration if:

- They are mentally alert and physically capable
- They understand the purpose, dosage, route, and timing of the medication
- They have previously been self-administering the medication at home (e.g., insulin, inhalers)
- No history of non-compliance or misuse

B. Authorization

- The treating consultant must evaluate and approve self-administration.
- An entry must be made in the **patient's medical record**, indicating approval and any special instructions.
- Nursing staff must complete an **initial assessment form** documenting patient capability.

C. Medication Control

- Medications for self-administration must be:
    - Prescribed by a registered medical practitioner
    - Stored safely (e.g., bedside locker or secure container)
    - Properly labeled with instructions
- Narcotics or high-risk medications **must not** be self-administered unless exceptional approval is provided.

D. Nurse's Role

- Educate the patient on correct administration technique, dose, and timing.

- Monitor compliance, especially in the initial period.
- Record in the **Medication Administration Record (MAR)** that the dose was self-administered.
- Evaluate periodically to ensure continued safety.

E. Documentation

- Maintain clear records of:
  - Authorization for self-administration
  - Education provided
  - Any errors or missed doses
  - Periodic reassessments

F. Monitoring and Audits

- Pharmacy and nursing team to perform regular checks on:
  - Medication storage
  - Usage pattern
  - Adherence to timing and dose
- Any deviation or adverse event must be reported to the treating physician and quality department.

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6. Safety Precautions

- Self-administration is **revoked immediately** if:
  - The patient is found to be non-compliant or confused
  - An error or adverse event occurs
  - The clinical condition changes